

COXAN™ TABLET

Description : White circular, flat beveled-edge, uncoated tablet marked on one side with letter 'M' on top half and 'PL' on bottom half, other side marked 'PARACETAMOL'.

Content

Each tablet contains paracetamol 500mg.

Indications

For the relief of fever and pain.

Pharmacology

Paracetamol has analgesic and antipyretic effects. It may produce analgesia by inhibiting prostaglandin synthesis in the central nervous system and through a peripheral action by blocking pain impulse generation. The peripheral action may also be due to inhibition of the synthesis of prostaglandins or to inhibition of the synthesis or action of other substance, which pain receptors to mechanical or chemical stimulation sensitize. Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat regulating centre to produce peripheral vasodilatation resulting in increased blood flow through the skin, sweating and heat loss. The central action probably involves the inhibition of prostaglandin synthesis in the hypothalamus.

The oral absorption of paracetamol is rapid and almost complete and may be decreased if taken after a high carbohydrate meal. Protein binding is not significant with doses producing plasma concentration below 60mcg/ml but may reach moderate level with high or toxic doses. Approximately 90% - 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulphuric acid and cysteine. The half-life is 1 - 4 hours and dose not change with renal failure but may be prolonged in some forms of hepatic disease, the elderly and neonates, and may be somewhat shortened in children. The duration of action is 3 - 4 hours. Excretion is renal, primarily as conjugates and 3% of a dose maybe excreted unchanged.

Adverse effects

Side effects of paracetamol are usually mild, though hematological reactions have been reported.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Steven Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Precautions

Paracetamol should be given with care in patients with hepatic disease, viral infection impairment and to patients taking other drugs that affect the liver.

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time.

Prolonged use except under medical supervision can be harmful.

If symptoms persist for more than 3 days, consult the doctor.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash.

These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Drug interaction

Alcohol, hepatic enzyme inducers, hepatotoxic medication - risk of hepatotoxicity with single toxic dose or prolonged use of high doses of paracetamol.

Anticoagulant, coumarin - or indandione - derivative-chronic concurrent high dose paracetamol may increase the anticoagulant effect.

Non-steroidal Anti-inflammatory analgesics (NSAIDs) - prolonged concurrent use of paracetamol may also increase the risk of adverse renal effects.

Dosage and Administration

Adults and children over 12 years : 1 - 2 tablets three to four times daily.

Not more than 8 tablets in 24 hours unless instructed by a doctor.

Children 6 - 12 years : 1/2 - 1 tablet three to four times daily

Children under 6 years : Not recommended.

Treatment of overdose

Signs of overdose may appear shortly after ingestion and persist for 24 hours. These include diarrhoea, loss of appetite, nausea or vomiting, stomach cramps or pain and an unusual increase in sweating. Signs of possible liver damage, such as abnormalities in liver function tests, pain or tenderness in the liver area, enlargement of the liver or jaundice may occur within 2 - 4 days after ingestion. Maximal changes in liver function tests usually occur 3 - 5 days after ingestion. Hepatic encephalopathy, convulsions, respiratory depression, coma cerebral edema, coagulation defects, gastrointestinal bleeding, disseminated intravascular coagulation hypoglycemia, metabolic acidosis, renal tubular necrosis, cardiac arrhythmias and cardiovascular collapse may occur. Empty stomach via induction of emesis or gastric lavage. If activated charcoal has been administered, it should be removed by lavage before the antidote is given. Administer N-acetylcysteine orally (140mg/kg body weight initially then 70gm/kg body weight every 4 hours for 17 doses), diluted to 5% solution with cola, grapefruit juice, orange or water. If necessary the antidote may be given via duodenal intubation. N-acetylcysteine is most effective if treatment is started within 24 hours. Supportive treatment consists of maintaining fluid and electrolyte balance, correcting hypoglycaemia and administering vitamin K fluid (if prothrombin ratio time exceeds 1.5) and fresh frozen plasma or clotting factor concentrate (if prothrombin time ratio exceed 3.0).

Storage : Keep container tightly closed. Store in a cool, dry place below 30°C. Protect from light.

Presentation : 10 x 10's /box, 100 x 10's/ box.

Shelf-life : 3 year.

Manufactured For:



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